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AMA policies to ensure AI supports—not replaces—physician judgment

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CHICAGO — Physicians and medical students at the Annual Meeting of the American Medical Association (AMA) House of Delegates adopted new policies to ensure AI strengthens patient care, supports evidence-based medicine, and remains under the oversight of physicians rather than replacing physician judgment.

The policies address the growing use of AI in both clinical decision support and health insurance coverage determinations, establishing principles that AI should serve as an assistive tool—not an autonomous decision-maker—and that transparency, accountability, and physician oversight are essential whenever AI is used in patient care.

“AI has enormous potential in healthcare, but it cannot replace physician judgment,” said AMA CEO John Whyte, MD, MPH. “Patients deserve care decisions that are informed by the latest medical evidence and guided by a physician who understands their individual needs.

Whether AI is helping a physician make a clinical decision or assisting with an insurance review, there must always be transparency, accountability, and meaningful physician oversight.

Technology should support better care – not stand between patients and the care they need.”



ensure that coverage decisions are based on accurate, up-to-date, evidence-based medical information and are reviewed by physicians with appropriate expertise.

The new policy opposes the use of autonomous or semiautonomous AI systems as substitutes for physician review in coverage determinations and calls for regulations that require AI-enabled technologies to be integrated into physician-led processes. The AMA also will advocate for greater transparency when AI is used in prior authorization and other utilization management decisions, including disclosure of the clinical logic, data sources, and guidelines used to reach adverse determinations.

“When health plans use AI-driven tools to deny or delay care without explaining how those decisions were reached, physicians and patients are left in the dark,” Dr. Whyte said. “AI should never function as an unaccountable black box. Health plans must be transparent about how these tools work, what evidence and data sources they rely on, and whether a qualified physician reviewed the decision.”

To strengthen accountability, the AMA will advocate for regular audits of AI-driven clinical review tools, including audits triggered by significant changes to AI models, training data, or clinical guidelines, as well as comprehensive annual reviews to ensure continued alignment with current standards of care.

The AMA also adopted policy aimed at ensuring AI clinical decision support tools reflect the principles of evidence-based medicine and provide physicians with information they can understand, evaluate, and trust. Under the policy, the AMA will work with medical specialty societies, regulators, AI developers, and other stakeholders to establish standards for evidence attribution, evaluation, validation, transparency, and explainability in AI-enabled clinical decision support systems.

While AI-enabled tools may improve efficiency and help synthesize large amounts of information, the AMA notes that important concerns remain regarding transparency, bias, explainability, and long-term impact on physician practice and patient outcomes.

The AMA will continue its work through the AMA Center for Digital Health and AI to ensure physicians help shape how AI is developed, implemented, and regulated across the health care system, with patient safety and physician-led care remaining at the center of those efforts.

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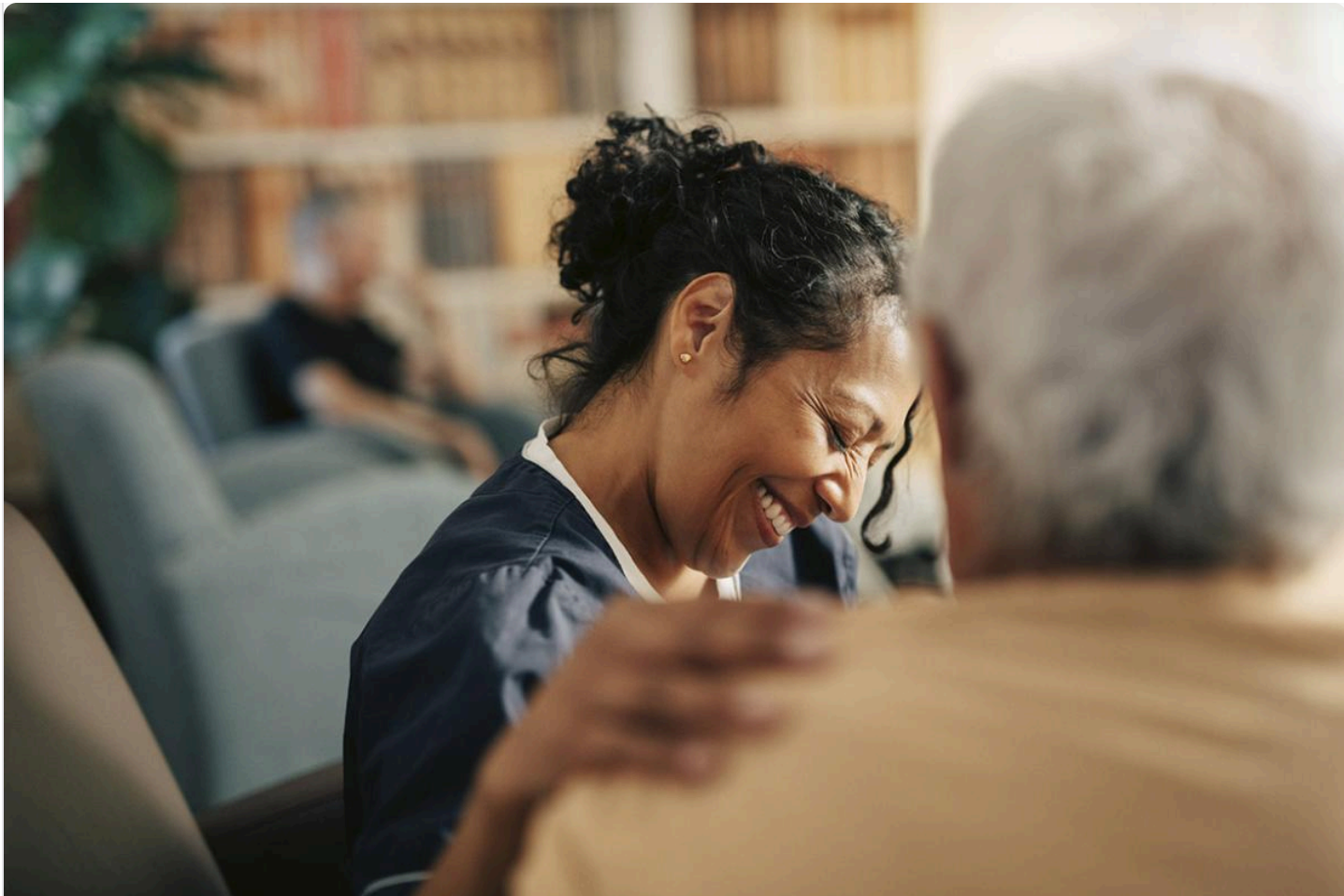
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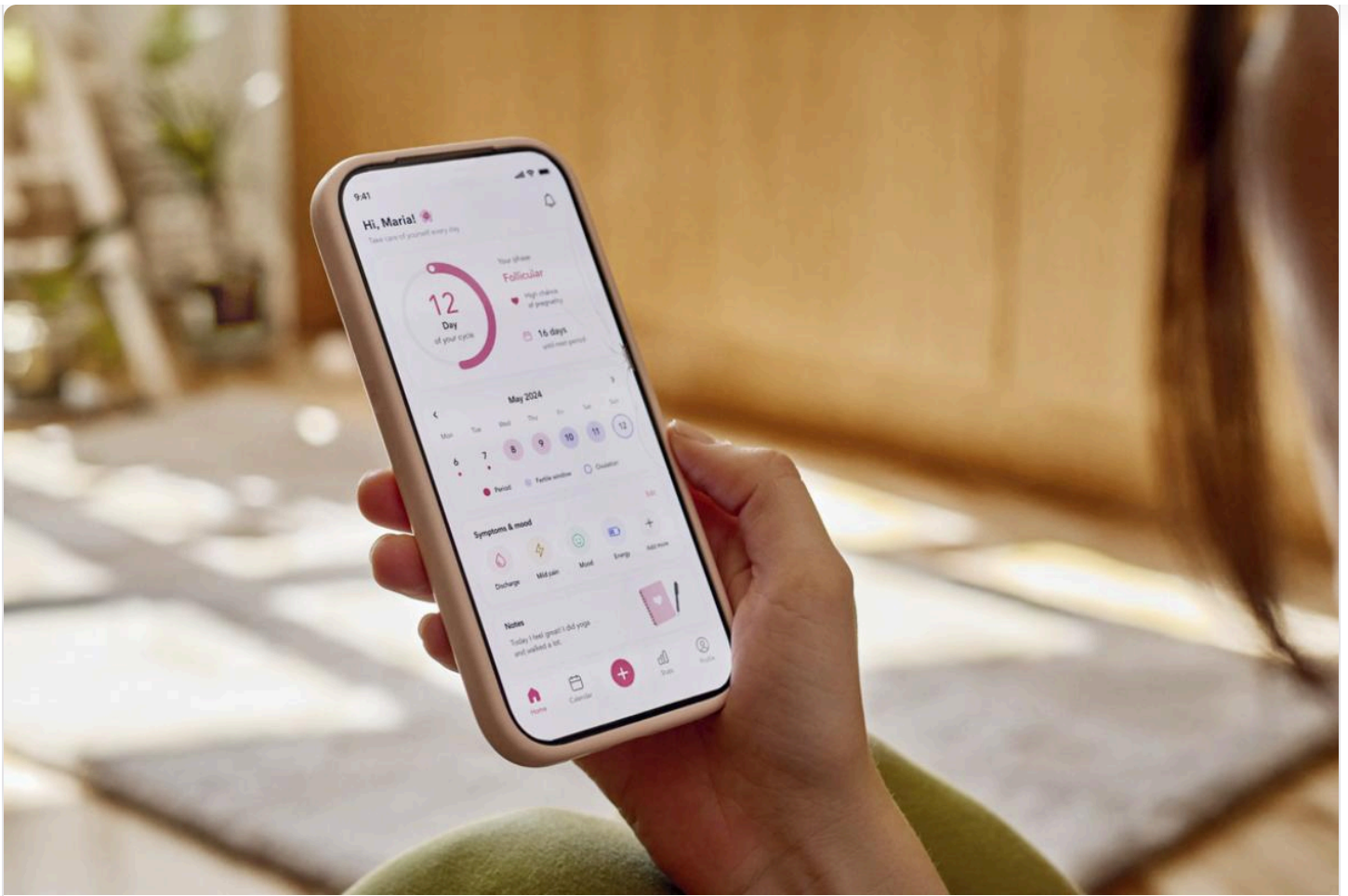
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